

OPERATIVE REPORT

St. Ambrose Medical Center – 400 Chew St, Allentown PA

Patient: Cole, Jasmine DOB 02/14/1997 Date of surgery: 08/12/2024

Surgeon: M. Okafor, MD, FACS Assistant: L. Tran, PA-C

Anesthesia: General endotracheal

PREOPERATIVE DIAGNOSIS: Strictureing ileocecal Crohn's disease with recurrent partial small bowel obstruction, refractory to medical therapy.

POSTOPERATIVE DIAGNOSIS: Same.

PROCEDURE PERFORMED: Laparoscopic ileocecal resection with primary stapled side-to-side (functional end-to-end) ileocolic anastomosis; lysis of adhesions (approx 25 minutes); APPENDECTOMY performed as part of the ileocecal specimen.

FINDINGS: 12 cm segment of thickened, strictured terminal ileum with creeping fat and a fibrotic stricture 5 cm proximal to the ileocecal valve. Moderate omental and pelvic adhesions. Cecum grossly involved at the valve. No perforation, no abscess, no skip lesions palpated in the proximal small bowel (run from ligament of Treitz).

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TECHNIQUE (SUMMARY): 4-port laparoscopic approach. Adhesiolysis as above. Ileocolic pedicle divided with vascular stapler. Terminal ileum divided 3 cm proximal to diseased segment; ascending colon divided at the cecal-ascending junction. Specimen (terminal ileum, cecum, and APPENDIX en bloc) removed via Pfannenstiel extraction site. Side-to-side stapled ileocolic anastomosis created; common enterotomy closed. Anastomosis widely patent, pink, no tension, no leak on inspection.

ESTIMATED BLOOD LOSS: 50 mL. SPECIMEN: Ileocecal resection including appendix, to pathology. COMPLICATIONS: None.

PATHOLOGY (ADDENDUM 08/19/2024): Chronic active ileitis with transmural inflammation and fibrostenosis, consistent with Crohn's. Margins free of active disease. Appendix with fibrous obliteration, no acute inflammation. 14 lymph nodes, reactive.

DISPO: Discharged POD 3, uncomplicated. Plan postoperative biologic prophylaxis per GI (adalimumab + azathioprine started 10/2024).